



INTEGRATED COMMUNITY PAEDIATRIC CARE IN A DISADVANTAGED SETTING

A RETROSPECTIVE REVIEW OF THE KIDSCOPE CLINIC INTEGRATED CARE MODEL

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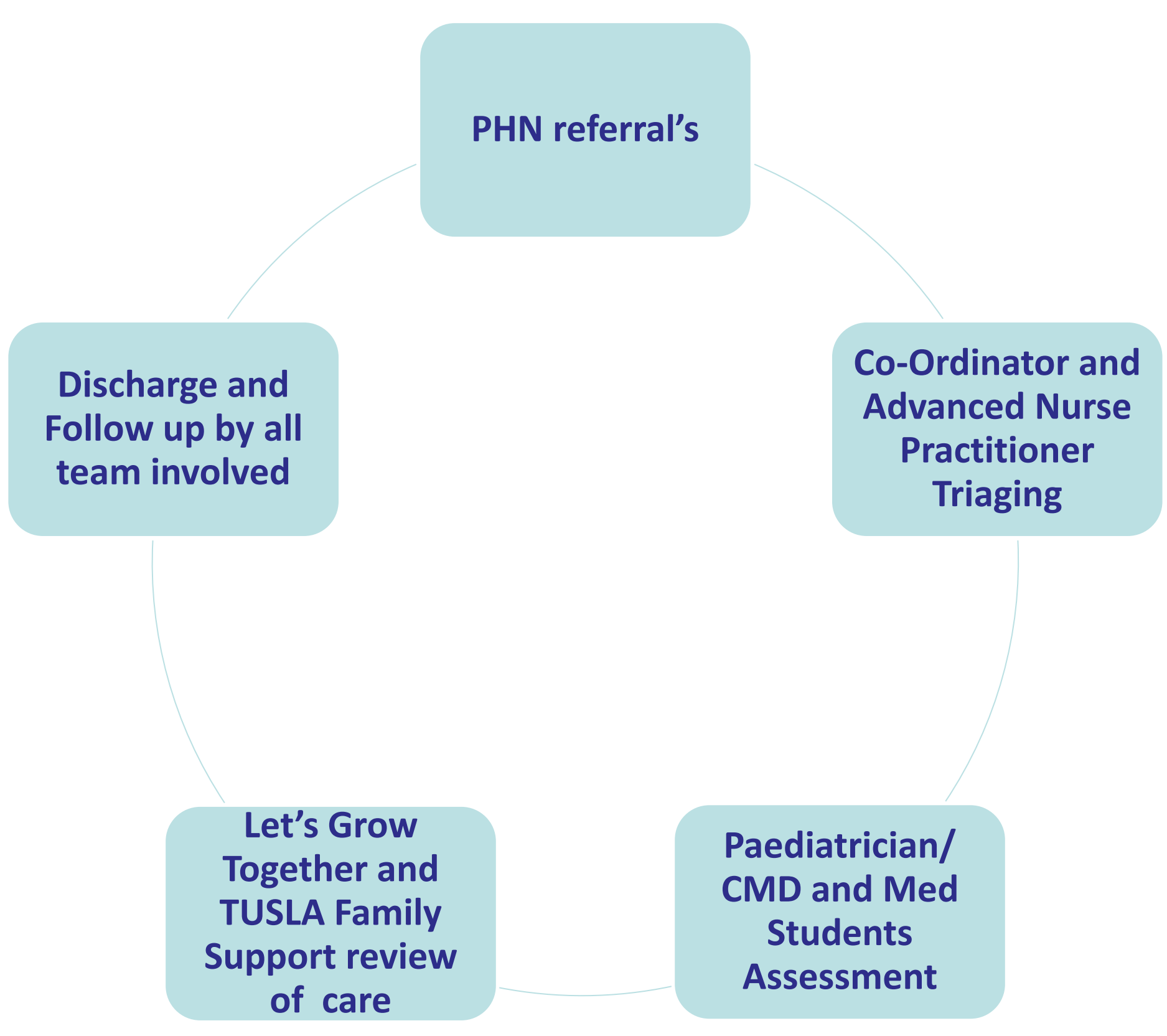
Introduction

Children living in disadvantaged communities often have complex developmental and healthcare needs, requiring multidisciplinary support. The **Kidscope** Paediatric Development Clinic in Cork was set up as a teaching clinic for medical students, providing an integrated model of care linking healthcare, education, and community services. This operation enables the next generation of healthcare professionals in Ireland to have clinical experience in a community setting from a social inclusion perspective.

Background and Objectives

This study aimed to examine the clinical activity within each profession and determine how collaborative working can support access to assessment, intervention, and family-centred care.

Fig 1.1 Flowchart of integrated care in action at the Kidscope Clinic



Methodology

An analysis of each role was undertaken, following a retrospective review of clinic activity over two academic years. Data collected included attendance, referral reasons, multidisciplinary involvement, audiology screening activity, and clinical outputs. Collaboration with community support services were also examined. Professionals in clinic include a Paediatrician, community medical doctors, medical students, Let's Grow Together, audiologists, social inclusion ANP, public health nurses and TUSLA Family support workers.



Let's Grow Together
The best start for every child
Community Ref: 458905 | Clinic Ref: 30200676 | 017 22495

Let's Grow Together provides community-based prevention and early intervention supports that promote child development and family wellbeing. The **Let's Grow Together** team act as a relational bridge between the clinic, families and clinical and community services. Families are supported to follow recommendations and engage with home visiting, parent-and-toddler and parent-only groups.



TUSLA

Tusla Family Support provides early intervention and practical support to promote positive outcomes and wellbeing. They are part of the **Kidscope** review meetings to help identify children and families who may benefit from additional support. They provide direct links to family support services and share knowledge of local community resources, promoting a coordinated approach to care.



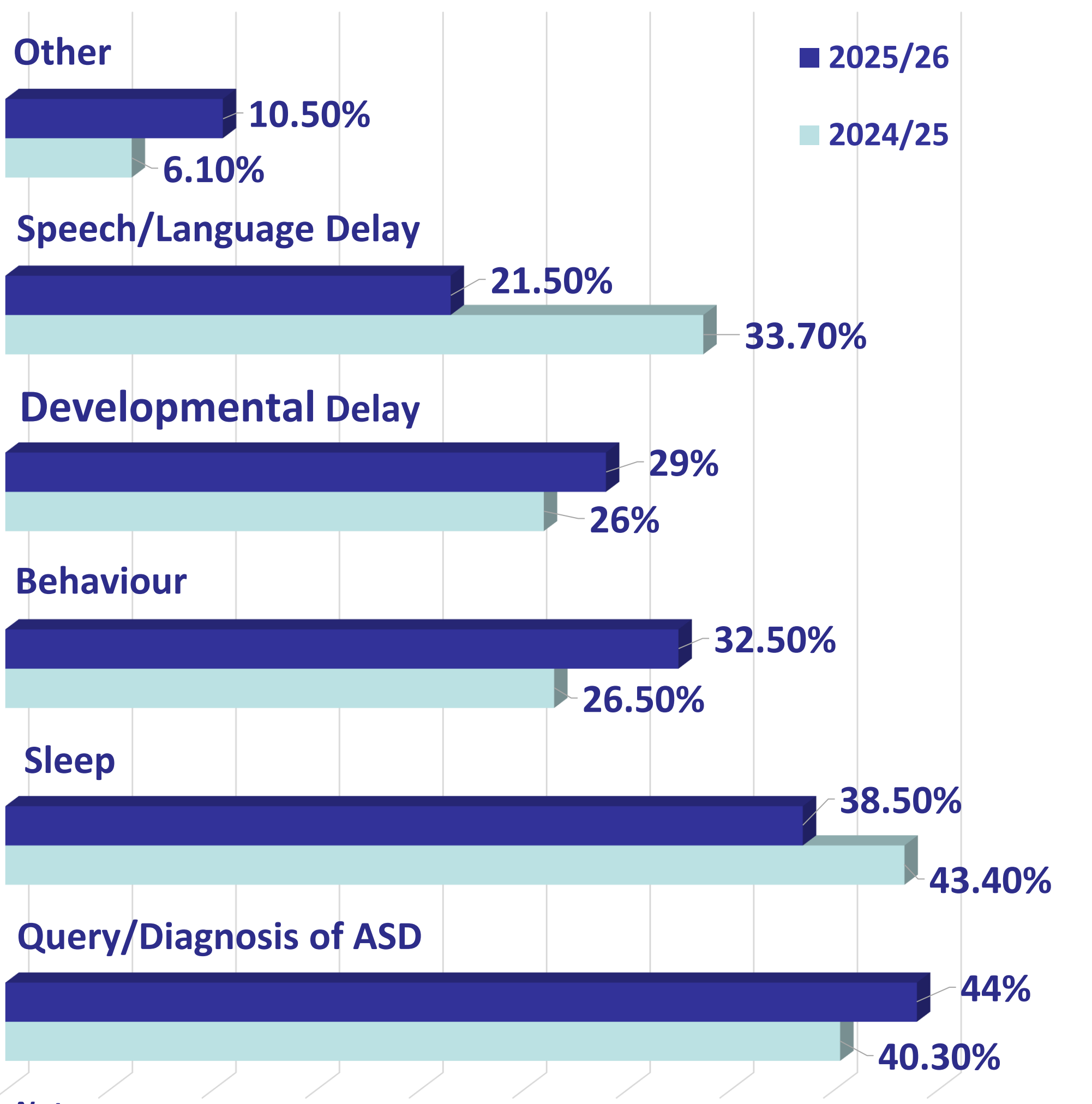
NICHE
HEALTH PROJECT CORK CLG

Kidscope is based in the **Niche Family Health Project community centre**, which provides paediatric care in an accessible, local setting. Locating the clinic within an established community allows Kidscope to deliver care closer to home and strengthen connections with families living in disadvantaged areas.

Results

Between 2025 and 2026, thirty-five clinics were held from a multidisciplinary team, delivering 200 appointments for 148 children, with an 86% attendance rate. Common referrals included developmental delay, speech and language concerns, behavioural difficulties, sleep issues, and social deprivation. Involving more professionals increased multidisciplinary involvement and generated more support for children and families.

Fig 1.2 Referral reasons from PHN's/LGT in 2025/26



Note
Graph is based off the 200 appointments attended in which 52 of these are repeat appointments
Referrals can have more than one concern
Other includes Diet, Sensory issues, dysmorphic features, daily vomiting, gross motor delay, Intellectual Disability, Cerebral Palsy, High BMI and poor balance

Working at Kidscope from a community medical doctor perspective

“Although CMDs frequently refer and seek expert advice from paediatricians, **Kidscope is the only clinic in HSE Southwest where a CMD works alongside a Consultant Paediatrician and other community partners.** It allows CMDs to provide an enhanced service – for example recommending medication, discussing children who will need investigations without referring for a separate paediatric OPD consultation, using MDT discussion to provide parenting and social support etc. It is an excellent learning opportunity for CMDs, as well as strengthening ties between CMDs and the Paediatric Service.

A senior audiologist working at the Kidscope Clinic

Audiology provides an additional opportunity to identify hearing concerns early and direct children into appropriate services (redirecting to South Infirmary Hospital or St Finbarr's Hospital in Cork) as required. In this setting, there is also potential for increased interprofessional learning between medical and audiology students. In 2026, **Kidscope** provided a ‘drop in appointment’ service for children registered with the clinic, to allow more children be screened.

Fig 1.3 Pilot screening at Kidscope in partnership with audiology UCC

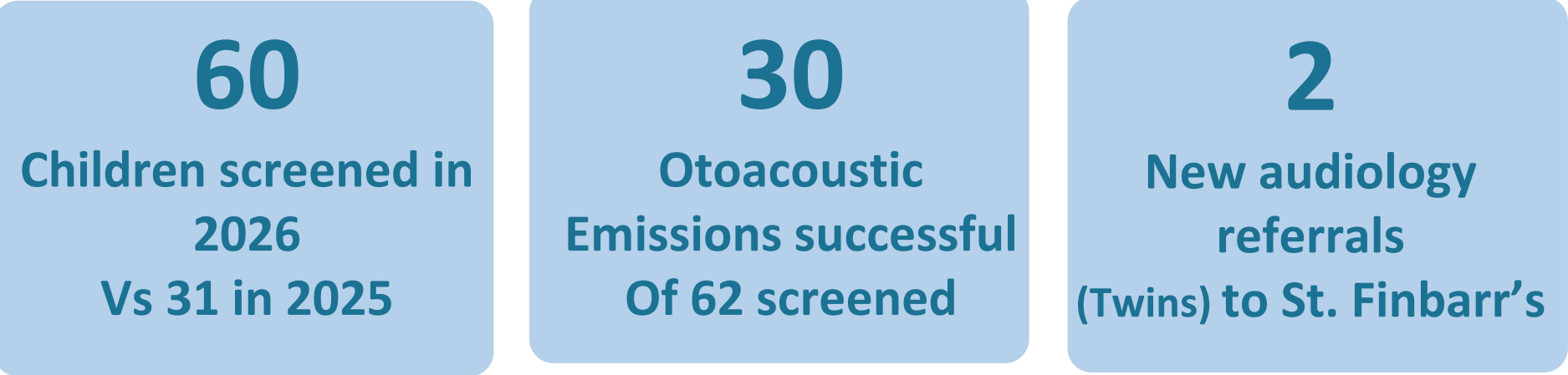


Fig 1.4 Audiology outcome breakdown (n=60)

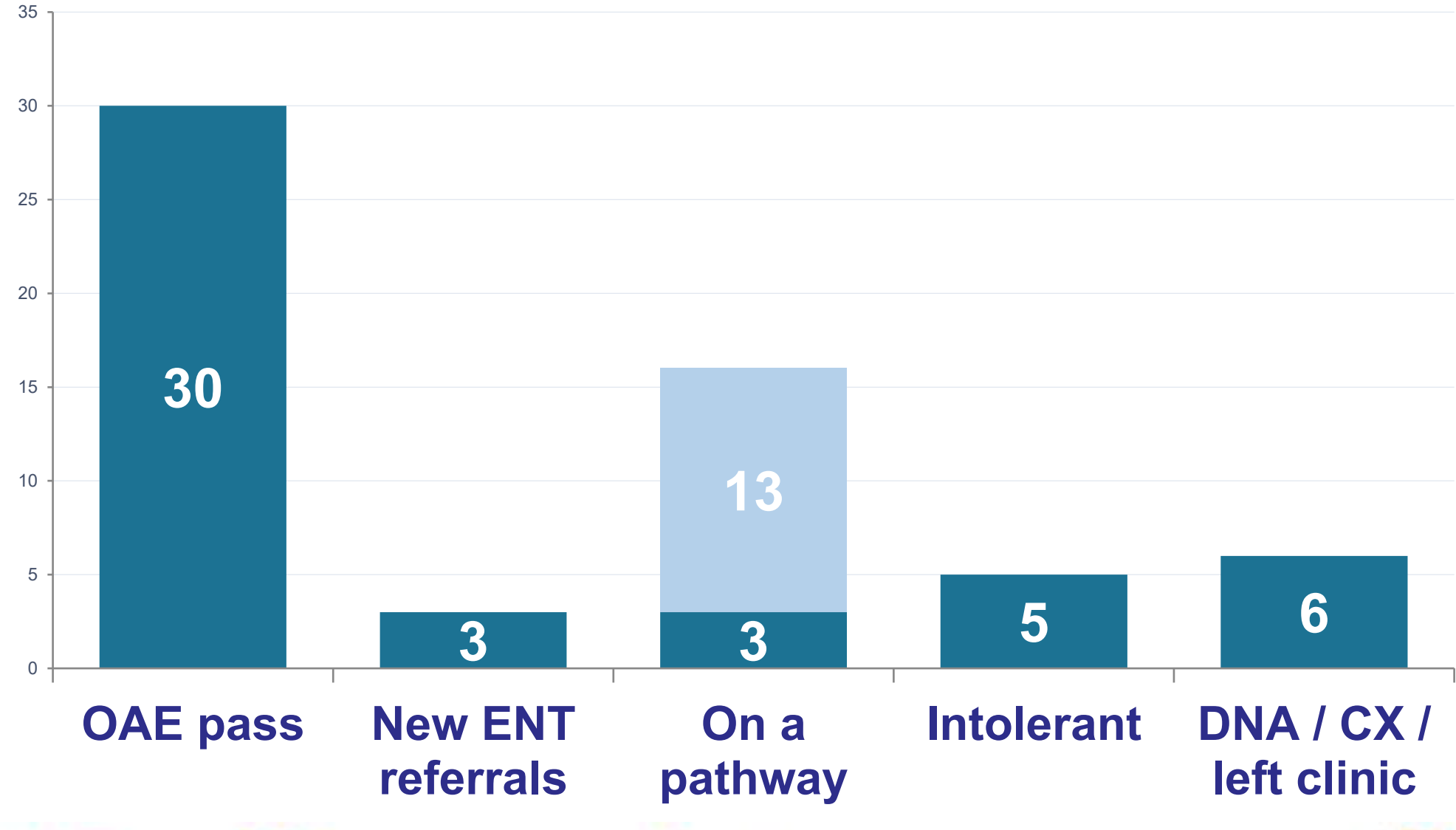
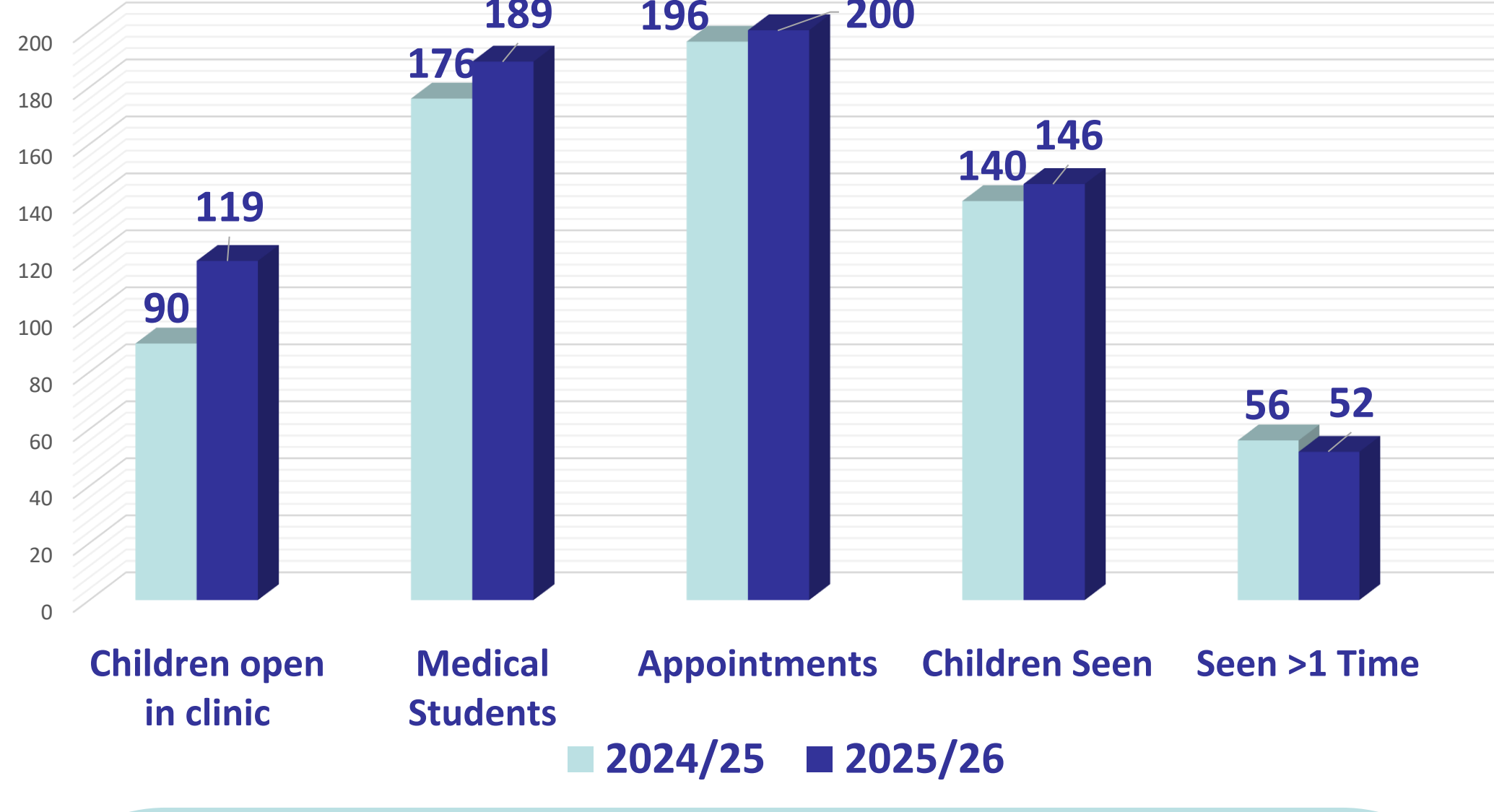


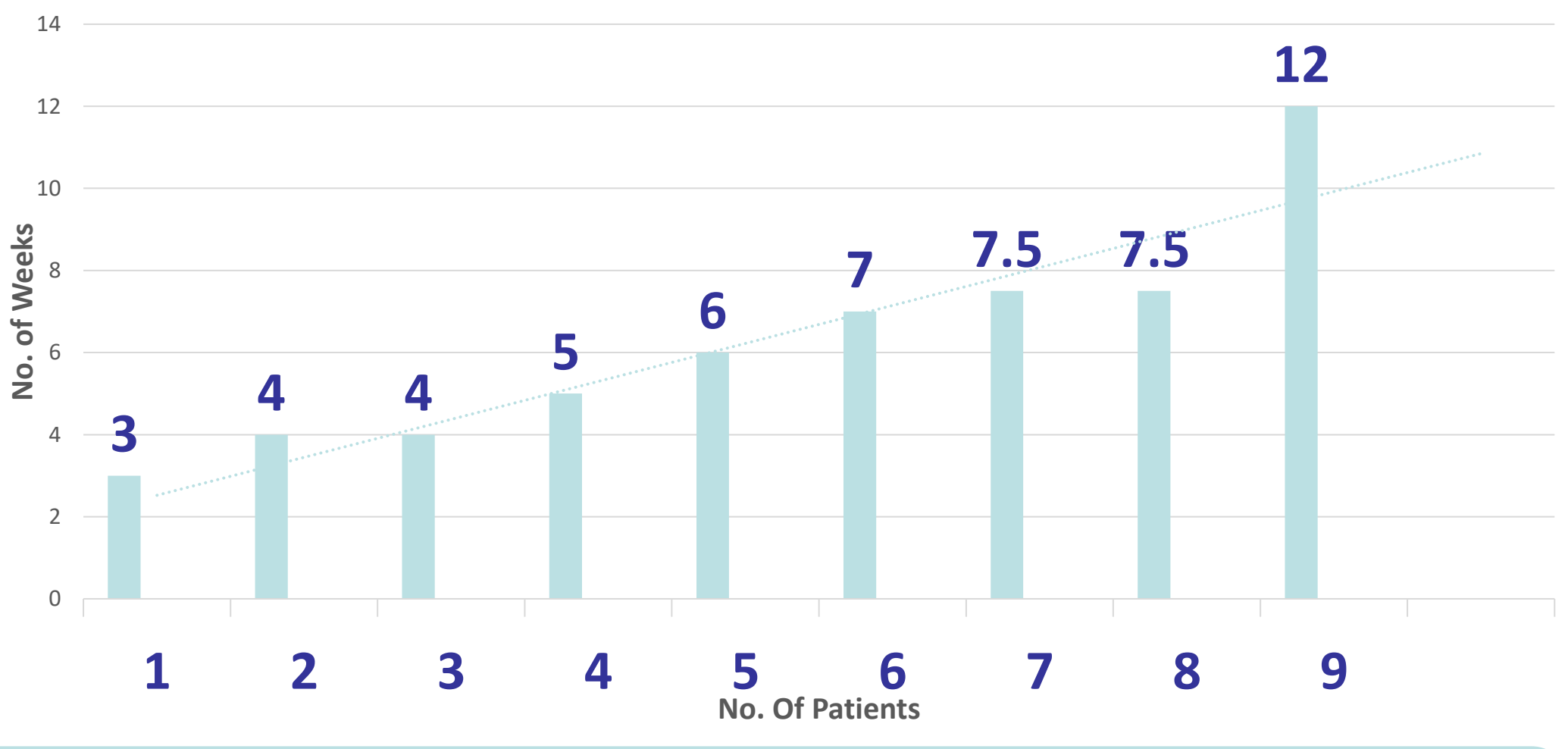
Fig 1.5 Comparison of clinic activity across two academic years with more multidisciplinary team involvement



Working at Kidscope from an advanced nurse practitioner perspective

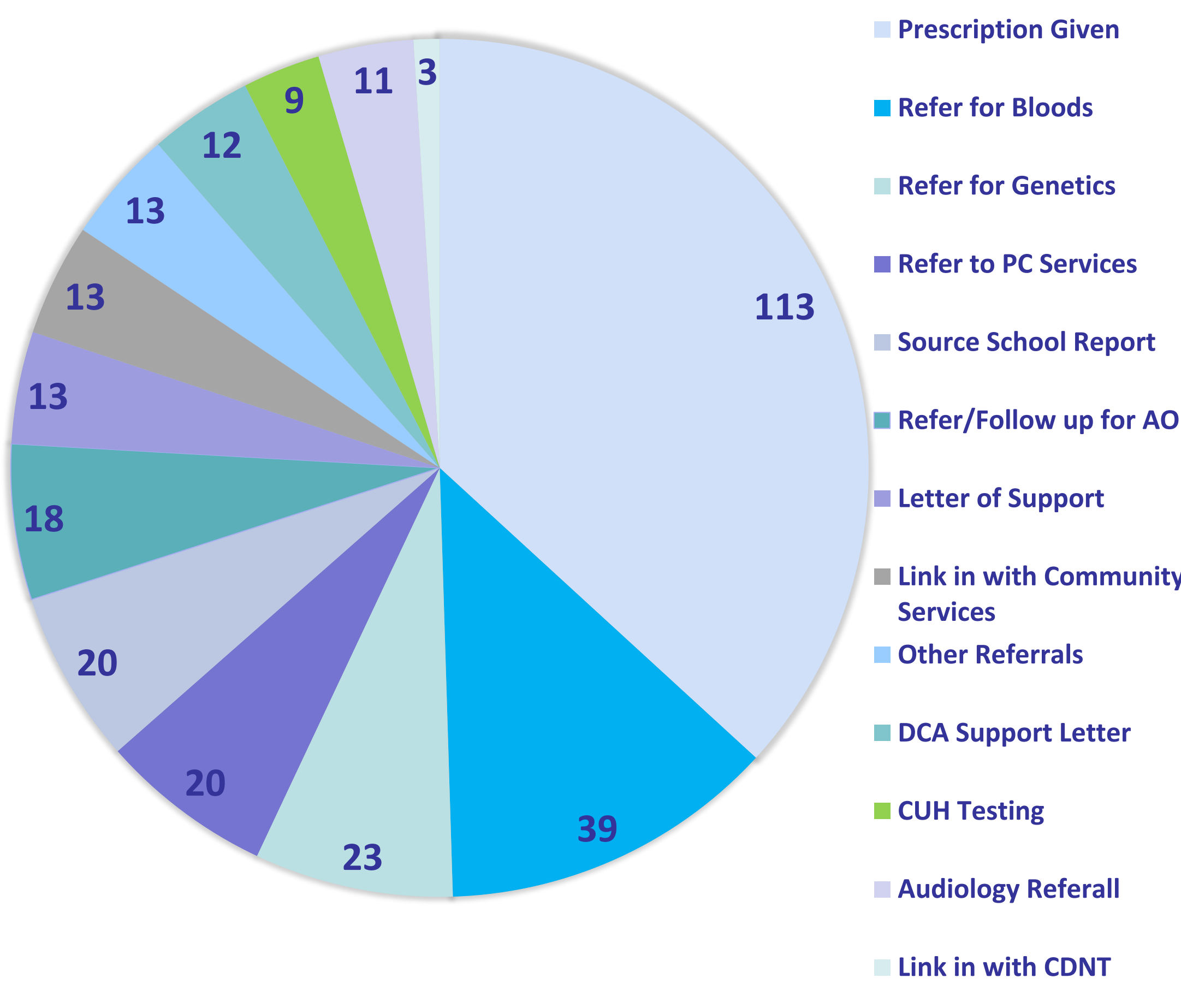
“The Advanced Nurse Practitioner (ANP) service provides specialist assessment, intervention, and care coordination for infants and young children (0–6 years) experiencing complex health and developmental needs associated with homelessness, parental substance use, and social disadvantage. The service works collaboratively with Public Health Nursing, primary care, maternity, and community services to deliver trauma-informed, family-centred care within community settings. A key component of the service is the **Kidscope Paediatric Social Inclusion Clinic**, where the ANP works as part of a multidisciplinary team alongside paediatricians and allied health professionals to provide integrated assessment, early intervention, and coordinated care for children experiencing health inequities.”

Fig 1.6 Referral to first appointment waiting time in weeks (N=9 children living in homeless accommodation between 2024 to 2026)



The average waiting time for a new referral from Homeless accommodation to be seen was 6 weeks (average waiting time for a new referral to be seen in Cork University Hospital in Developmental Pediatrics is 43 weeks)

Fig 1.7 Outputs from clinical collaboration



Conclusion

The **Kidscope** model highlights the value of multidisciplinary partnerships in reducing barriers to care and supporting earlier intervention. Bringing professionals from health, education, and community services together within a single assessment pathway enables coordinated decision-making and holistic care planning tailored to each child's needs. This multidisciplinary approach helps improve service experience for children and their families. These findings support the development of similar integrated community-based services to improve equity, service coordination, and outcomes for children and families.



For more info please contact the Kidscope clinic co-ordinator on 086 801 2360



Delivering Integrated Care Together